

12.7 cm

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LF-0150

BALTIC®

Composition :

Each mL contains :

Terbutaline Sulfate 0.5 mg
Benzyl Alcohol 20.0 mg

Pharmacodynamics :

Terbutaline sulfate is directly acting sympathomimetic amine with bronchodilator action and is used similar to those of salbutamol. It is used in the treatment of asthma, chronic bronchitis, emphysema, and other bronchopulmonary disorders involving bronchospasm.

Pharmacokinetics :

Absorption	Rapidly absorbed after subcutaneous administration.
Blood concentration	Subcutaneous administration of Terbutaline Sulfate (0.5 mg dose) shows peak plasma concentration level 7.6 µg / L in 15-30 minutes
Half-life	Serum half-life, 3 to 4 hours.
Distribution	Small amounts are secreted in the milk.
Metabolic Reactions	Subject to first pass metabolism mainly by sulphate conjugation, glucuronic acid conjugates are formed to some extent but the drug does not appear to be methylated.
Excretion	After an intravenous or subcutaneous dose, up to about 90% may be excreted in the urine, mostly as unchanged drug.

Indications :

For the relief of bronchospasm in bronchial asthma, chronic bronchitis, emphysema and other bronchopulmonary diseases where bronchoconstriction is a complicating factor.

Dosage and Administration :

½ ampoule (0.25 mg) subcutaneous injection 1-4 times a day or as directed by the physician.

Contraindications :

During the 1st and 2nd trimester of pregnancy. In premature labour associated with toxemia of pregnancy or antepartum hemorrhage.

Precautions :

1. In children and young patients.
2. In patients taking other drugs that can also cause extrapyramidal reactions eg. phenothiazines.
3. Care should be taken with patients suffering from myocardial insufficiency or thyrotoxicosis.
4. Anticholinergic and narcotic antagonize effects of Terbutaline and effects of CNS depressants may be enhanced.

Side effects :

Nausea, vomiting and headache. Tremor, tonic cramp, palpitation, tachycardia and arrhythmias.

Overdosage (Symptoms and Treatment) :

After use of terbutaline transient irritation may occur. Nausea and headache have been reported. Overdosage or accidental administration by mouth may cause depression of the central nervous system with marked reduction of body temperature and symptoms of drowsiness and coma, particularly in children. Treatment of side-effects is symptomatic. Symptoms including bradypnea, cold damp skin, somnolence, and tachycardia occurred in a boy after taking an emulsion of terbutaline. Most symptoms had disappeared 10 hours later but tachycardia persisted for 4 days.

Drug Interactions :

1. Other sympathomimetic broncodilators or epinephrine should not be used concomitantly with terbutaline sulfate.
2. Terbutaline sulfate should be administered cautiously to patients being treated with monoamine oxidase inhibitors or tricyclic antidepressants, since the action of Terbutaline sulfate on the vascular system may be potentiated.
3. Beta-receptor-blocking agents and Terbutaline sulfate inhibit the effect of each other.
4. β-adrenergic receptor blocking agents not only block the pulmonary effect of terbutaline but may produce severe asthmatic attacks in asthmatic patients.

Warning :

This preparation contains benzyl alcohol, its use should be avoided in children under two years of age. Not to be used in neonates.

As maternal pulmonary oedema and myocardial ischaemia have been reported during or following premature labour in patients receiving beta2 - agonists, careful attention should be given to fluid balance and cardiorespiratory function, including ECG monitoring. If signs of pulmonary oedema and myocardial ischaemia develop, discontinuation of treatment should be considered.

Due to the risk of pulmonary oedema and myocardial ischaemia that has been observed during the use of beta2 - agonists in the treatment of premature labour, before these products are given to any patient with known heart disease, an adequate assessment of the patient's cardiovascular status should be made by a physician experienced in cardiology.

Cautious use of salbutamol/terbutaline injections is required in pregnant patients when it is given for relief of bronchospasm so as to avoid interference with uterine contractility. During IV infusion of salbutamol/terbutaline the maternal pulse should be monitored and not normally allowed to exceed a steady rate of 140 beats per minute.

Tocolysis : Serious adverse reactions including death have been reported after administration of salbutamol/terbutaline to women in labor. In the mother, these include increased heart rate, transient hyperglycaemia, hypokalaemia, cardiac arrhythmias, pulmonary oedema and myocardial ischaemia. Increase fetal heart rate and neonatal hypoglycaemia may occur as a result of maternal administration.

Storage condition : Store in a cool dry place below 30°C.

Reg. No. : MAL19950511AZ

Shelf-life : 4 years

Pack size : 50 ampoules of 1 mL

Description : Clear colourless sterile solution.

Date of Revision : 6 September 2021

Manufactured by :



L.B.S. LABORATORY LTD., PART.

602 Soi Panichanant, Sukhumvit 71 Rd., Bangkok 10110 Thailand.

Product Registration Holder :

HEALOL PHARMACEUTICALS SDN. BHD.

74-3, Jalan Wangsa Delima 6, KLSC Wangsa Maju, 53300 Kuala Lumpur, Malaysia.

FRONT

BACK

Code : R 036/21
Project : LF F&B Baltic Inj.
Date : 06-09-2021
Size : 12.7x18 cm (WxH)
Colour : **K=100**

Export Malaysia

R036-21_Baltic Inj_LF F&B_ML_1stDraft_SM_06-09-2021

18 cm

18 cm